جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : احمد مختار ابراهيم مختار
المحافظة : الجيزه **الحي / المدينة :** البدرشين **العنوان :** 0
رقم البطاقة : 30201172104495 **رقم التليفون :** 01010377951 **تاريخ تسجيل الإستمارة :** 19-10-2026
العمر(تاريخ الميلاد) : 17-01-2002 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع:(----------)

الإخصائي الاجتماعي
التوقيع:(----------)

التشخيص الإكلينيكي : اورام ليمفاويه
نتائج الأبحاث : (مرفق)
التشخيص النهائي : اورام ليمفاويه
خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي ونقل دم ومشتقاته [13491]
قسم العلاج : الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: احمد مختار ابراهيم مختار
National ID: 30201172104495

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of lymphoma for Supportive and palliative Treatment and blood transfusion/blood products

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



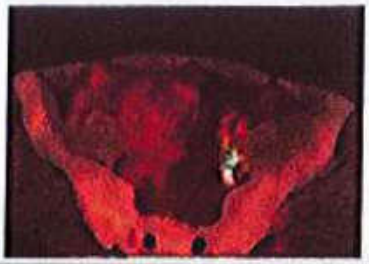
موجه اخلاقيه



Name : Ahmed Mokhtar Ibrahim Mokhtar	Date : 05-04-2026 08:44
Branch : Dokki	Medical No. : 342641
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستقلون

NILE DIAGNOSTIC CENTER

Abdomen and Pelvis:







- The surgical bed shows underlying thickening reaching 21mm of maximum thickening with SUV max of 5
- A soft tissue sheet like is noted with calcifications seen just below the described thickening at the right iliac fossa measuring 5x5 cm extending along the pelvic wall sides encasing the right lower end of the urethra with subsequent right sided hydro-ureto-nephrosis reaching SUV max of 2.2 , the sheet is seen also surrounded the prostate
- Enlarged prostate with a metabolic active focus seen on the right posterior peripheral zone reaching SUV max of 6.2
- No evidence of hepatic or splenic FDG avid focal lesions.
- No metabolically active pelvi-abdominal LNs noted.
- There are no FDG avid adrenal nodules.
- No evidence of FDG avid peritoneal nodules
- Physiologic FDG excretion is seen in the kidneys and bl[illegible]

Musculoskeletal:

- Normal marrow activity is seen along the surveyed skeleton
- No sclerotic or lyric lesions

Name: احمد مختار ابراهيم مختار	
MR. : 1125	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		**رقم طبي:**	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : | **تاريخ وساعة الخروج :**

وبعد الكشف والفحوصات والمتابعة وجد انه

RECURRENT, METASTATIC RIGHT SIDE COLON CANCER. The patient is receiving Avastin 5 mg/kg and Irinotecan 290 mg every 2 weeks. On examination, there is right lower quadrant pain and tenderness with diffuse abdominal tenderness, consistent with abdominal pain and tenderness (APUS).

The patient is a 24-year-old male with recurrent colon cancer, mucinous adenocarcinoma signet ring type. In 2023 he underwent right hemicolectomy with peritoneal biopsy revealing mucinous adenocarcinoma metastatic T4aN2B. He received FOLFOX + XELOX for 8 months but discontinued due to DPD intolerance. In 8/2025 a bladder neck mass of mucinous adenocarcinoma was noted; PET-CT was negative for DPD homozygous mutation. MDT on 3/12/2025 recommended FOLFIRI with dose modification per DPD deficiency. The patient received 2 cycles of FOLFIRI and developed rectal bleeding, severe chest pain, and fatigue. On 31/12/2025 the patient was started on Irinotecan + Avastin (C4 Camptox 290 mg, C3 Avastin 5 mg/kg, height 175 cm, weight 59 kg, BSA 1.69 m², B 831, TLC 5.9 ×10⁹/L, PLT 424 ×10⁹/L) and received 1 PRBC.

C4 Camptox 290 mg, C3 Avastin 5 mg/kg, height 175 cm, weight 59 kg, BSA 1.69 m².

The patient is being treated for metastatic rectal cancer with Avastin 400 mg IV, Campto 300 mg IV, Leucovorin 200 mg IV, and 5-FU 2 g on a 44-hour pump.

Recurrent colon cancer with locally advanced disease and peritoneal metastases. PET-CT 4/2026 shows thickening of the operative bed and peritoneal deposits. The patient is continuing FOLFIRI with the same dose and Avastin for symptom control of diarrhea.

Today the patient is receiving FOLFIRI + Avastin 400 mg IV without Campto as it is not available. The regimen includes Avastin 400 mg IV, Leucovorin 200 mg IV, and 5-FU 2 g on a 44-hour pump.

For FOLFIRI and Avastin, the patient had bilateral hydronephrosis and underwent double J stent placement for new pain. An erect X-ray showed constipation.

The patient has grade 3 diarrhea and right psoas muscle inflammation. Broad-spectrum antibiotics were omitted per Dr. Mohammed Ismaeel's advice. Campto 280 mg IV and Avastin 390 mg IV (weight 52 kg, height 175 cm, BSA 1.58 m²) were administered.

The patient is feverish, with severe vomiting and dehydration. He has psoas muscle inflammation versus hematoma on new CT. The patient is dehydrated with metabolic acidosis (pH 7.3) and is referred for emergency review.

تحريرا في ,

التاريخ : 21-09-2026 **الوقت :**

الطبيب المعالج : dina makram

يعتمد ,

Abdominal ultrasound shows a large collection in the left perinephric region measuring 18.9 × 9.4 cm, pending PET-CT.

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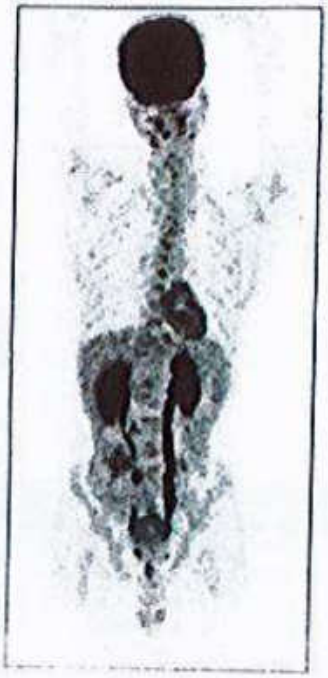
NILE DIAGNOSTIC CENTER

Whole Body F18- FDG PET/CT study

Clinical history:- 22 year old male patient , presenting with operated cancer colon referred for PET CT study

Type of the study :follow up study compared to old film dated 7/10/2025

rocedure:



- **Radiopharmaceutical:** 18F FDG, 3.2 mCi I.V.
- **Blood glucose level at time of 18F-FDG injection:** 79 mg/dL .
- **Time from 18F-FDG injection to scan:** 60 min
- **Technique :**MDCT diagnostic examination was taken for attenuation correction anatomic localization followed by PET images from the skull vault to the mid thigh were obtained.
- Images of CT and corresponding functional PET images are taken in axial, coronal and sagittal planes.
- **Patient's height: 170 cm patient's weight:60kg**
- **SUV average of reference hepatic activity=1.4 .**
- **Measurements are correlated to body weight.**

PET/CT FINDINGS:-

Head and Neck:

- No metabolically active cervical LNs noted.
- The brain exhibited normal FDG bio-distribution
- Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.
- Normal thyroid uptake

Chest:

- Normal FDG uptake is seen throughout the both lung lobes with no FDG avid lung nodules or masses detected.
- There are no pleural or pericardial abnormalities.
- Physiologic FDG uptake is noted in the myocardium.

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NILE DIAGNOSTIC CENTER

Conclusion:

- First PET/CT study reveled metabolic active mural thickening at the surgical bed with metabolic active soft tissue sheets seen as described for correlation with the pervious MRI and histopathological correlation suggestive of recurrence
- Enlarged prostate with metabolic active right peripheral zone for TRUS and PSA correlation

Thanks for referral;
Dr. NORA NABIL , MD
Lecturer of Radio-diagnosis,
ASUH

D M> ١١٢٥

NILE SCAN & LABS

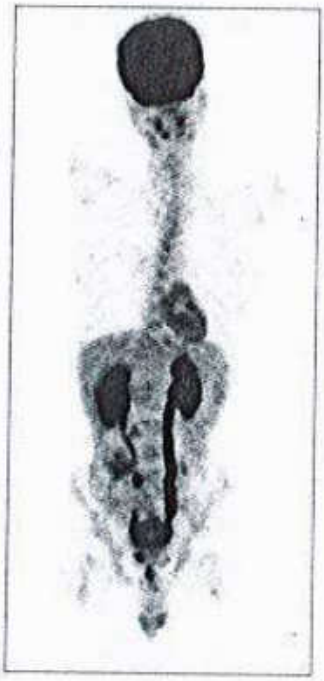
مركز النيل للأشعة والتحاليل

Name : Ahmed Mokhtar Ibrahim Mokhtar	Date : 05-04-2026 08:44
Branch : Dokki	Medical No. : 342641
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستشفيات

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NILE DIAGNOSTIC CENTER

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Thanks for referral;
Dr. NORA NABIL , MD
Lecturer of Radio-diagnosis,
ASUH

NILE DIAGNOSTIC CENTER

Visit Number:	71125526571	Client Name:	نقابة المهن الزراعيه
Patient Name:	احمد مختار ابراهيم	Referred by Prof:	-
Age / Sex:	23 year / Male	Exam Date:	08 Nov 2025
Patient ID:	2138989625	Authenticated Date:	08 Nov 2025 11:30:25 PM
		Printed:	09 Nov 2025 04:47:52 AM

MRI

intermediate T2 signal, highdiffusion signal and moderate post contrast enhancement.

- The forementioned soft tissue lesion is seen distending the prostatic urethra / bladder neck forming a 3.0x4.5x3.5 cm polypoidal mass lesion protruding into urinary bladder neck.
- Perirectal fat smudging; however normal MR appearance of the rectum.
- Mildly enlarged liver showing homogenous texture and regular smooth outlines. No definite focal lesions or IHBRs dilatation.
- Patent homogenously enhancing main portal vein and its central branches.
- Average sized spleen showing no definite focal lesions.
- Normal MR appearance of the pancreas, adrenal glands, left kidney, aorta and IVC.
- No ascites.







Conclusion:

Reported case of ascending colonic adenocarcinoma received adjuvant therapy and underwent surgery. (no available operative details or pervious studies).

- Diffuse thickening of the anastomotic site with multiple omento peritoneal nodularities / masses; as well as pelvic wall / mesorectal creeping soft tissue lesion

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		Printed:	09 Nov 2025 04:47:52 AM

MRI

invading seminal vesicles, prostatic urethra and extending within urinary bladder neck. (suggestive of tumoral recurrence with peritoneal deposits and locally advanced pelvic neoplastic growth).

- For follow up, kindly compare with the previous unavailable films.
- *For further PET CT evaluation.*

Dr. Evram E.Bishoy, MD

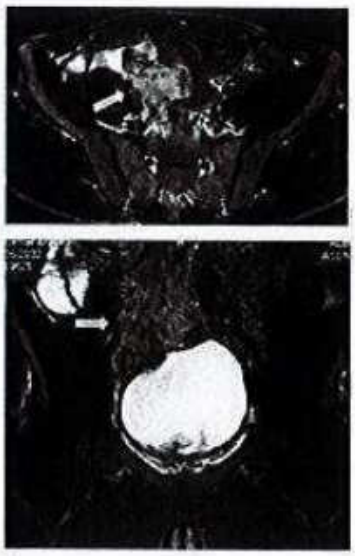
Dr. Evram E.Bishoy, MD

Visit Number:	71125526571	Client Name:	نقابة المهن الزراعيه
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MRI

- Behind anterior abdominal wall at right lumbar region 2.1x4.8 cm.
- Subhepatic 1.6x1.7 cm
- Perihepatic along surface of segment VI, 1.3x2.9 cm.
- Right iliac fossa region, measuring 3.9x5.4x5.9 cm, seen medial to right iliac vessels, abutting the urinary bladder dome and entangling / encasing the pelvic course of the right ureter. Subsequent mild fullness of the right renal collecting system.

➢ Creeping sheet like soft tissue lesion along the lateral pelvic walls, mesorectal fascia, infiltrating both seminal vesicles, ejaculatory ducts. It displays



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Visit Number:	71125526571	Client Name:	نقابة المهن الزراعيه
Patient Name:	احمد مختار ابراهيم	Referred by Prof:	-
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MRI

MRI OF THE ABDOMEN & PELVIS WITH IV CONTRAST

Technique:

- **Multiplanar multisequences study before and after IV contrast administration with Diffusion images and ADC.**

CLINICAL HISTORY: *Ascending colon cancer since 2022 underwent surgical excision and chemotherapy (Biopsy revealed mucinous adenocarcinoma), Now complains of dysuria and hematuria, UB endoscopy and biopsy revealed also mucinous adenocarcinoma.*

FINDINGS:

No available operative details or previous studies for comparison.

- The right lumbar ileocolic anastomotic site shows mild diffuse wall thickening with multiple surrounding omento peritoneal nodularities / enhancing soft tissue masses with tiny calcifications ***as follow:***







Name	**Mr. Ahmed Mokhtar Ibrahim Mokhtar**		
Ref. By Prof. Dr.	**Mohamed Kotb**	**Date**	7/10/2025
ID	1123030319	**Acc. No.**	291283795

^{18}F-FDG PET/CT Scan

Indication of the study: Status follow-up study for a case with previous history of managed colonic malignancy. No available pathological data. Compared to 9.2023.

Total body PET scan was done 40 minutes after IV administration of 300 MBq of 18F-FDG.

CT scanning was performed simultaneously without and with IV contrast material injection

Semi-quantitative analysis would be done for selected ROI.s & the normal threshold would be the normal blood pool liver $SUV_{max.} < 2$

Reported By:

Dr. Islam Aboulwafa, MD

Approved By:

Prof. Dr.

Essam El-Shiekh

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain.
- (N.B; not all brain metastatic lesions could be detected on 18-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-/oro-pharynx, larynx, thyroid and salivary glands.
- No enlarged FDG hypermetabolic cervical L.Ns seen.

Torso:

Thorax:

- Bilateral tracer starved atelectasis and sub-pleural reticulations.
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.

Abdomen & Pelvis:

- ***No sizable bowel active masses.***
- Few peri-sutural strandy reticulations of no evident uptake, mostly post management sequelae, for follow up.
- Enlarged liver with non-uniform activity yet no FDG hypermetabolic focal lesions, mostly blood pooling. For dynamic MRI study of the liver assessment if warranted.
- No biliary tree dilatation.
- Average size of the spleen with no focal active lesions.
- Normal appearance of pancreas (no sizable active masses), adrenals and pelvic organs.
- No enlarged FDG hypermetabolic abdominal or pelvic L.Ns seen.
- Congested mesentery with strandy reticulations with low activity peritoneal reflections thickening.

Name	**Mr. Ahmed Mokhtar Ibrahim Mokhtar**		
Ref. By Prof. Dr.	**Mohamed Kotb**	**Date**	7/10/2025
ID	1123030319	**Acc. No.**	291283795

- Normal physiological biodistribution over the bowel (non-uniform bowel uptake with no sizable masses) & urinary tracts.

Surveyed skeleton:

- Faint marrow uptake of SUVmax. 2.5 yet no tracer active lytic or sclerotic osseous lesions seen, likely hematopoietic, for CBC correlation and follow up.

Opinion:

- The surveyed body is free from any FDG hypermetabolic neoplastic masses.
- Tumor markers levels correlation would be recommended.

Thanks for your referral;

Dr. Ahmed Khodeir
MD Nuclear Medicine

Dr. Islam Aboulwafa, MD
Prof. Dr. Essam Ali El-Shiekh

NAME: Ahmed Mokhtar	DATE : 2/9/2025.
Referred by Prof. Dr.: Ahmed Moustafa Sror	

PelviAbdominal ultrasound

⇒ *Enlarged prostate with estimated volume about 36ccs with central zone seen hypertrophied seen invaginating bladder base with intravesical small component seen showing irregular surface, no calcifications noted*

⇒ *Dilated seminal vesicles in both sides more at RT side showing heterogenous texture*

⇒ *Adequate UB filling showing turbid content with precipitated milk of calcium noted, no definite stones seen*

⇒ *Residual urine post micturition measures about 98ml*

⇒ *Liver: average in size with homogenous texture and no evidence of focal lesions. Portal vein is not dilated. C.B.D. is not dilated and no intrahepatic biliary dilatation. Normal appearance of portal tracts and hepatic veins.*

⇒ *Spleen: average size 11cm with homogenous echo pattern and no focal lesions.*

⇒ *Gall bladder: contracted with no evidence of stones seen inside*

⇒ *Pancreas and para- aortic area appear normal.*

⇒ *No ascites.*

⇒ *Both kidneys: Normal size, contour and anatomical position (RT 9.5x3.5cm, LT 10x4cm). Parenchyma is preserved showing normal echogenicity. No calculi, cysts, masses or back pressure changes.*

CONCLUSION:

- ***Enlarged prostate with bladder base invaginations as described with residual urine as described for further work up***
- ***Dilated seminal vesicles***
- ***Turbid urine with precipitated milk of calcium seen within***

Much Obliged

Dr. Ahmed Darka, FEBR

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : احمد مختار ابراهيم مختار
Analysis ID : 756557
Age : 23 Y : 8 M : 4 D
Gender : Male
Referred By Prof. Dr. : ياسر محمد
Clinical Diagnosis : Cecal mass
Nature of Specimen : Right hemicolectomy & Peritoneal nodules biopsy
Receiving Date : 2023-06-13
Patient ID : 578558

PATHOLOGY REPORT

Gross :

Two designated containers were received:
[1] Right hemicolectomy specimen formed of a segment of colon measured about 15 cm long with end measured 3 cm in diameter and an ileal loop measured 7 cm long with end measured 2 cm in diameter. The appendix not detected.
On opening, there was an ulcerated infiltrating mass, measured 7x5x4 cm, infiltrating the pericolic fat and serosa.
The tumor located 7 cm away from colonic margin.
Dissection of the pericolic fat revealed 42 lymph nodes, the largest measured 1.5x1 cm.
[2] Peritoneal nodules: Two cauterized flat tissue pieces, measured 1.5x1.3 cm and 0.7x0.5 cm, multisected and totally submitted.

Microscopic :

Sections examined from the specimen [1] received revealed colonic wall with infiltrating tumour formed of hyperchromatic malignant columnar cells arranged in signet ring cells 40%. There is extracellular mucin 60%.
Lympho- vascular space invasion present.
Perineural invasion present.
There is no tumoural necrosis with minimal desmoplasia.
Tumour tissue extensively infiltrating the pericolic fat and infiltrating serosa.
Surgical margins are free.
Examination of the adjacent lymph nodes revealed metastatic deposits in 7 out of 42 lymph nodes with extranodal extension.
Sections examined from the specimen [2] received revealed metastatic peritoneal deposits.

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : 25744742 - 25773253 - 25790935 - 01287005217

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
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PATHOLOGY REPORT

Diagnosis :

Cecal mass, Right hemicolectomy, INFILTRATING MUCINOUS ADENOCARCINOMA WITH SIGNET RING CELL DIFFERENTIATION, FREE SURGICAL MARGINS, POSITIVE LYMPH NODE METASTASIS, L.N 7/42 WITH EXTRANODAL EXTENSION.
[2] Peritoneal nodules, Biopsy, METASTATIC PERITONEAL DEPOSITS.
- TUMOR STAGE: pT4a, pN2b.

Copy

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : 01287005217 - 25790935 - 25773253 - 25744742

Patient Name	: الأنسه / احمد مختار ابراهيم	Request No.	: 13000773
Patient No.	: 30201172104495	Request Date	: 02/11/2025
Path No.	: SR-1546/25	Verification Date	: 08/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 23 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Urinary bladder neck mass in known case of cancer cecum.
Nature of specimen: Referred blocks & slides (TURBT, Complete resection).

Gross examination:
Two referred blocks & two referred (H&E) stained slides were received labelled by (993454).

Microscopic examination
- Histopathological examination reveals transitional epithelial lining with cystitis glandularis overlying tumor formed of excess extra-cellular mucin entangling malignant epithelial cells showing signet ring cell morphology with peripherally located hyperchromatic nuclei and intracytoplasmic mucin. Tumor infiltrates lamina propria. No muscle proper identified.

Diagnosis
Urinary bladder neck mass, Referred blocks & slides (TURBT, Complete resection):
Mucinous adenocarcinoma with signet ring cell differentiation, infiltrating lamina propria.
- Correlate with clinical history & ragiologic findings.

Signature
Dr. Walaa Ibrahim Ghoneim.
Dr. Duaa Mohamed.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Name	**Mr. Ahmed Mokhtar Ibrahim**		
Ref. By Prof. Dr.	**Ahmed Mostafa Abd Al-Salaam**	**Date**	25/9/2023
ID	1123030319	**Acc. No.**	291081914

^{18}F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of managed colonic malignancy. No available previous studies or pathological data.

Total body PET scan was done 40 minutes after IV administration of 300 MBq of 18F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROI.s & the normal threshold would be the normal blood pool liver *SUV*$_{max}$ < 2

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain.
- (N.B; not all brain metastatic lesions could be detected on 18-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-/oro-pharynx, larynx, thyroid and salivary glands.
- No enlarged FDG hypermetabolic cervical L.Ns seen.

Torso:

Thorax:

- Bilateral tracer starved atelectasis and sub-pleural reticulations.
- No FDG hypermetabolic pulmonary nodules or masses seen.
- No enlarged FDG hypermetabolic hilar or mediastinal L.Ns seen.
- No pleural or pericardial effusion seen.

Abdomen & Pelvis:

- *No sizable bowel active masses.*
- Few peri-sutural strandy reticulations of no uptake, mostly post management sequelae, for follow up.
- Enlarged liver with non-uniform activity yet no FDG hypermetabolic focal lesions, mostly blood pooling. For dynamic MRI study of the liver assessment if warranted.
- No biliary tree dilatation.
- Average size of the spleen with no focal active lesions.
- Normal appearance of pancreas (no sizable active masses), adrenals and pelvic organs.
- No enlarged FDG hypermetabolic abdominal or pelvic L.Ns seen.
- Congested mesentery with strandy reticulations.

Reported By:

Dr. Islam Aboulwafa, MD

Approved By:

Prof. Dr.

Essam El-Shiekh

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : احمد مختار ابراهيم مختار
Analysis ID : 993454
Age : 23 Y : 8 M : 4 D
Gender : Male
Referred By Prof. Dr. : احمد شعراوى
Clinical Diagnosis : Bladder mass around bladder neck
Nature of Specimen : TURBT (Complete resection)
Receiving Date : 2025-09-22
Patient ID : 578558

PATHOLOGY REPORT

Gross :

Multiple irregular soft greyish tan tissue pieces, measured collectively 4x4 cm, totally submitted.

Microscopic :

Sections examined from the specimen received revealed bladder wall with hyperplastic polypoidal transitional lining with proliferation, Brunn's nests, infiltrated by tumor tissue formed of malignant columnar cells with hyperchromatic nuclei showing signet ring differentiation, floating within extracellular mucus. Tumor extensively infiltrating the lamina propria and muscle layer.

Diagnosis :

Bladder mass around bladder neck, TURBT (Complete resection), MUCINOUS ADENOCARCINOMA WITH MARKED SIGNET RING DIFFERENTIATION, EXTENSIVELY INFILTRATING THE BLADDER WALL, POLYPOIDAL PROLIFERATIVE CYSTITIS.
- PLEASE CORRELATE WITH PREVIOUS PATHOLOGY.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

This report is electronically signed , any abrasion or modification would render it void .

عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٤٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧